

2024, Mar. 2024 Release LOC:Inpatient Rehabilitation**Medically Intensive Rehabilitation**Utilization Benchmarks: *Multiple options available, see table below for options***Overview****Preadmission****Severity of Illness****Admission, Both:****Severity of Illness****Intensity of Service****Continued Stay****Intensity of Service****Discharge Screens** ⁽¹⁰⁰⁾**Discharge** ⁽¹⁰⁰⁾**Utilization Benchmarks****Notes**

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Overview**Level of Care Note**

These criteria are designed for screening and review of patients with trauma, progressive central nervous system conditions, cardiac disease, pulmonary disease, or oncology conditions who are receiving services in any acute inpatient rehabilitation setting. Patients who required non-skilled or custodial services prior to their recent illness, injury, or surgery may be appropriate for a lower level of care; therefore, secondary review is required to determine the appropriate rehabilitation level of care and to document pre-morbid baseline.

This criteria subset is appropriate to use when the following patient status and program requirements are met (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Miller and Forer, Pm r 2021, 13: 535-9; Forrest et al., Medicine (Baltimore) 2019, 98: e17096):

Patient Status:

- Determined to have significant functional and/or cognitive limitations requiring at least 2 therapy disciplines (1 of which needs to be physical therapy or occupational therapy)
- Medically stable with medical or surgical co-morbidities manageable and not requiring acute medical attention
- Expected improvement from rehabilitation intervention within a reasonable and predictable period of time

Program Requirements:

- Comprehensive rehabilitation program providing at least 15 hours per week of intensive skilled therapy services
- A high level of supervision by a rehabilitation physician and medical management is required to manage conditions related to the admitting diagnosis and comorbidities that may impact the ability to participate in intensive therapies
- Rehabilitation physician provides admission orders, assessment, oversight, and program coordination
- Rehabilitation physician visits face-to-face with the patient at least 3 times per week
- A non-physician rehabilitation practitioner may conduct 1 of the 3 required face-to-face visits beginning the second week of acute rehabilitation stay
- Rehabilitation physician assesses the patient (medically and functionally) and modifies the plan of care as needed

- Skilled rehabilitation nursing services on-site availability 24 hours per day
- Preadmission screening assessment completed by a rehabilitation professional
- Treatment plan developed within 4 days of the patient's admission
- Interdisciplinary goal-oriented and coordinated treatment by a rehabilitation physician, professional nursing, social worker and/or case manager, and rehabilitation therapists with specialized training, education, and/or certification
- Daily documentation of patient treatment interventions with weekly documentation of patient progress including evaluation of goal status, progress towards outcomes, and any modification to the treatment plan
- Interdisciplinary team meeting at least weekly, inclusive of ongoing comprehensive discharge planning
- Pharmacy and diagnostic services available

InterQual® criteria are derived from the systematic, continuous review and critical appraisal of the most current evidence-based literature and include input from our independent panel of clinical experts. The content is based on a variety of references which are cited at specific criteria points throughout the subset.

The Medicare specific content in this subset is based on the Medicare Benefit Policy Manual (Chapter 1 - Inpatient Hospital Services Covered Under Part A).

Severity of Illness

● Severity of Illness, All:

Clinical and Functional

● Medically intensive condition and, **One:** ⁽¹⁾– Medicare beneficiary requiring intensive inpatient rehabilitation ^(2, 3)● Non-Medicare beneficiary and illness, injury, surgery, or exacerbation within the last 30 days requiring intensive inpatient rehabilitation and, **≥ One:** ⁽⁴⁾● Cardiac disease or post cardiac surgery, **Both:** ⁽⁵⁾– Active comorbidity ⁽⁶⁾– Functional limitation ⁽⁷⁾● Ventilator management or weaning, **Both:** ⁽⁸⁾

– Tracheostomy tube in place

– Functional impairment ⁽⁹⁾– Congenital deformity or disorder (e.g., spina bifida, cerebral palsy, spinal muscular atrophy) ^(10, 11)● Malignant or metastatic disease, or benign neoplasm (excludes end-stage), **≥ One:** ^(12, 13)– Bone ⁽¹⁴⁾– Brain ⁽¹⁵⁾– Lung ⁽¹⁶⁾– Spine ⁽¹⁷⁾– Major or multiple trauma ^(18, 19)– Progressive or degenerative neuromuscular disorder ^(20, 21, 22)– Myopathy ⁽²³⁾⁽²⁴⁾ Systemic vasculitis with joint inflammation and failed outpatient or less intensive rehabilitation setting● Uncontrolled pain with neurologic or musculoskeletal etiology, **All:**– Failed outpatient physical or occupational therapy ≥ 2 trials ⁽²⁵⁾– Failed pharmacologic or nonpharmacologic treatment ≥ 2 trials ⁽²⁶⁾

– Inability to work or manage household responsibilities

● Impairment (new) with functional activity limitation and, **One:** ⁽²⁷⁾● Medicare beneficiary requiring assistance and, **≥ One:** ^(3, 28)– ADLs impairment ⁽²⁹⁾– Cognitive impairment ⁽³⁰⁾– Language, speech, or swallowing impairment ⁽³¹⁾– Mobility or motor impairment ⁽³²⁾– Respiratory impairment (includes ventilator assistance) ⁽³³⁾● Non-Medicare beneficiary requiring at least minimum assistance and, **≥ One:** ^(28, 34)– ADLs impairment ⁽²⁹⁾– Cognitive impairment ⁽³⁰⁾– Language, speech, or swallowing impairment ⁽³¹⁾– Mobility or motor impairment ⁽³²⁾– Respiratory impairment (includes ventilator assistance) ⁽³³⁾– Patient requires and can tolerate an intensive rehabilitation program ≥ 15 h/wk of skilled therapies ^(35, 36, 37, 38, 39)– Clinically stable ≥ 24 h ^(40, 41)● Cognitive, **Both:**

– Able to follow visual or verbal commands

– Willing and able to actively participate

– Full participation in therapeutic evaluation and intervention prior to transfer ⁽⁴²⁾– Preadmission assessment completed by licensed or certified clinician and approved by the rehabilitation physician ⁽⁴³⁾– Rehabilitation potential based on prior level of function ^(44, 45)● Therapy indicated, **≥ One:** ⁽⁴⁶⁾

– PT and OT

● OT and, **≥ One:**

- ST ⁽⁴⁷⁾
- Prosthetics/orthotics ⁽⁴⁸⁾
- PT and, ≥ **One**:
 - ST ⁽⁴⁷⁾
 - Prosthetics/orthotics ⁽⁴⁸⁾

Intensity of Service*(At Least Daily)*● **Intensity of Service, One:**

Admission Review

- Plan of care documented within 4 days and, **All**: ^(49, 50)
 - Treatment goals defined and therapy initiated ^(39, 51)
- Medical or psychosocial management, ≥ **One**: ⁽⁵²⁾
 - Bowel or bladder ^(53, 54, 55, 56)
 - Coping skills or adjustment to functional loss ⁽⁵⁷⁾
 - Medication (includes PO) ⁽⁵⁸⁾
 - Nutrition
 - Pain ⁽⁵⁹⁾
 - Spasticity ⁽⁶⁰⁾
 - Respiratory
 - Wound or skin care ^(61, 62)
- Care coordination initiated ^(1, 63, 64)

Continued Stay

- **Responder**, rehabilitation potential maximized and discharge expected, **All**: ⁽⁶⁵⁾
 - Medical and neurologic stability ^(66, 67)
 - Pain or spasm controlled or manageable ^(60, 68)
- Functional status, ≥ **One**:
 - Prior level of function achieved ⁽⁶⁹⁾
 - Ambulates ≥ 50 ft with supervision, modified independent, or independent ^(28, 70)
 - Modified independent mobility with wheelchair or supervised wheelchair use ⁽²⁸⁾
 - Functional plateau reached or minimal functional gains (ADLs or mobility) for at least one week ⁽⁷¹⁾
- **Partial responder**, and potential for significant improvement, **One**: ⁽⁷²⁾
 - Clinically stable, **All**:
 - Patient requires and can tolerate an intensive rehabilitation program ≥ 15h/wk of skilled therapies ^(35, 36, 37, 38, 39)
 - Therapy indicated, ≥ **One**: ⁽⁴⁶⁾
 - PT and OT
 - OT and, ≥ **One**:
 - ST ⁽⁴⁷⁾
 - Prosthetics/orthotics ⁽⁴⁸⁾
 - PT and, ≥ **One**:
 - ST ⁽⁴⁷⁾
 - Prosthetics/orthotics ⁽⁴⁸⁾
 - Measurable progress documented toward pre-established goals with gains sustained, ≥ **One**: ^(39, 44, 73, 74)
 - ADLs ^(29, 75)
 - Cognition or role function ^(30, 76)
 - Decreased pain with increased tolerance of functional activities
 - Functional mobility, ≥ **One**: ⁽³²⁾
 - Ambulation
 - Bed mobility
 - Stair negotiation
 - Transfers
 - Wheelchair

- Pulmonary function, $\geq$ **One:** ⁽⁷⁷⁾
 - Aerobic capacity
 - Endurance ⁽⁷⁸⁾
- Predetermined and reasonable time frame documented for goal achievement ⁽⁷⁹⁾
- Care coordination documented ^(1, 63, 64)
- Medical management and education, $\geq$ **One:** ⁽⁵²⁾
 - Bowel or bladder ^(53, 54, 55, 56)
 - Equipment use, safety, or instruction
 - Medication management or teaching
 - Post surgical precautions
 - Positioning, splinting, or orthotic use ⁽⁸⁰⁾
 - Skin protection, wound or scar management, or pressure relief techniques ⁽⁸¹⁾
- Medical instability (new onset) and decreased participation in therapy for $\leq 3d$ and, $\geq$ **One:** ^(82, 83, 84)
 - Bed rest due to suspected DVT ⁽⁸⁵⁾
 - Chest pain (excludes suspected or known cardiac etiology)
 - COPD exacerbation and increase in baseline treatment ⁽⁸⁶⁾
 - Exercise or activity program induced instability, $\geq$ **One:** ⁽⁸⁷⁾
 - Hypoglycemic symptoms requiring adjustment, $\geq$ **One:**
 - Insulin
 - Diet, parenteral, or enteral feedings
 - O_2 sat $\leq 91\%(0.91)$ and $<$ baseline with minimal exertion requiring supplemental oxygen ^(88, 89)
 - Sustained or prolonged symptoms requiring diagnostic evaluation, medication, or intervention, $\geq$ **One:**
 - Dyspnea
 - Syncope ⁽⁹⁰⁾
 - Presyncope ⁽⁹⁰⁾
 - Extensive diagnostic tests off premises
- Fatigue $\leq 24h$, $\geq$ **One:**
 - Chemotherapy or radiation related complication
 - Hemodialysis ⁽⁹¹⁾
 - Medication induced
 - Other invasive procedure ⁽⁹²⁾
 - Recent ambulance transportation
- Heart failure exacerbation ⁽⁹³⁾
- Heart rate 100-120/min, sustained ⁽⁹⁴⁾
- Temperature $\geq 100.4^\circ F(38.0^\circ C)$ and drug induced fever suspected, **Both:** ⁽⁹⁵⁾
 - New onset
 - Precipitating drug discontinued $\leq 3d$
- Infection, actual or suspected, **Both:**
 - Finding, $\geq$ **One:**
 - Temperature $\geq 100.4^\circ F(38.0^\circ C)$ ⁽⁹⁵⁾
 - WBC $\geq 12,000/cu.mm(12 \times 10^9/L)$ or $> 10\%(0.10)$ bands
 - Treatment, $\geq$ **One:**
 - Anti-infective (includes PO)
 - Culture pending $\leq 2d$
 - Imaging study $\leq 24h$
- Mental status change and neurological assessment $\geq 4x/24h$ ^(96, 97)
- Neurologic change and neurological assessment $\geq 4x/24h$ ^(97, 98)
- Oxygenation $<$ baseline and requiring supplemental oxygen, $\geq$ **One:** ^(88, 89)
 - O_2 sat $\leq 91\%(0.91)$ ^(88, 89)
 - Arterial PO_2 56-60 mmHg(7.5-8.0 kPa)
- Uncontrolled pain requiring new pain management regimen ⁽⁹⁹⁾
- R/O fracture after fall and imaging studies scheduled or pending $\leq 24h$

- Vomiting or diarrhea and IV fluids ≥ 50 mL/h
- **Orthostatic hypotension, Both:**
 - Sustained drop in blood pressure within 3 min of sitting or standing
- **Blood pressure, $\geq$ One:**
 - Systolic BP drop ≥ 20 mmHg
 - Diastolic BP drop ≥ 10 mmHg
- Wound dehiscence

Discharge Screens● **Discharge, One:** ⁽¹⁰⁰⁾**Clinical,**● **Home, All:**

- Home environment safe and accessible ⁽¹⁰¹⁾

● **Patient or caregiver, Both:**

- Demonstrates ability to manage transfers or functional mobility (e.g., ambulation, wheelchair), ADLs, or IADLs
- Independent with home therapy regimen ⁽¹⁰²⁾

● **Complete prior to discharge, All:**

- Follow-up care planned ⁽¹⁰³⁾
- Comprehensive written discharge and teaching instructions reviewed ⁽¹⁰⁴⁾
- Medication reconciliation ^(105, 106, 107)
- Patient or caregiver understands when and where to seek help
- Identify and address transportation needs

● **Home Care, All:**

- Home environment safe and accessible ⁽¹⁰¹⁾
- Patient or caregiver able to learn care ⁽¹⁰⁸⁾
- Treatment regimen established

● **Skilled services, $\geq$ One:**

- Clinical assessment ⁽¹⁰⁹⁾
- Medication management ⁽⁵⁸⁾
- Patient or caregiver education ⁽¹¹⁰⁾
- Therapy evaluation
- Wound or skin care ⁽⁶¹⁾

● **Complete prior to discharge, All:**

- Follow-up care planned and home care services arranged ⁽¹⁰³⁾
- Comprehensive written discharge and teaching instructions reviewed ⁽¹⁰⁴⁾
- Medication reconciliation ^(105, 106, 107)
- Patient or caregiver understands when and where to seek help
- Identify and address transportation needs

● **Skilled Medical or Therapy, All:**

- Medical practitioner, NP, or PA assessment or oversight ≥ 1 x/wk
- Treatment precluded at a lower level ⁽¹¹¹⁾

● **Skilled services, $\geq$ One:**● **Able to tolerate 1-2h/d of skilled therapy ≥ 5 d/wk, All:**

- Functional impairment requiring at least supervision ⁽²⁸⁾
- Goal directed therapy and at least 1 therapy discipline required
- Rehabilitation potential with expectation for clinical and functional improvement ⁽⁴⁵⁾

● **Skilled nursing service required daily, $\geq$ One:** ⁽¹¹²⁾

- Respiratory intervention ≥ 2 x/24h daily
- Nursing intervention or assessment 1-2x/24h
- Pain management or analgesic ⁽⁵⁹⁾
- Wound or skin care at least daily ⁽⁶¹⁾

● **Complete prior to facility transfer, All:**

- Comprehensive written discharge and teaching instructions reviewed ⁽¹⁰⁴⁾
- Medication reconciliation ⁽¹⁰⁵⁾
- Obtain and complete forms for facility
- Obtain discharge summary and transmit to facility and medical practitioner
- Arrange transportation
- **Subacute Medical or Therapy, All:**
 - Medical practitioner, NP, or PA assessment or oversight $\geq 2x/wk$
 - Treatment precluded at a lower level ⁽¹¹¹⁾
- **Skilled service, $\geq$ One:**
 - **Able to tolerate 2-3h/d of skilled therapy $\geq 5d/wk$, All:**
 - ≥ 2 functional impairments requiring at least minimum assistance ⁽²⁸⁾
 - Goal directed therapy and ≥ 2 therapy disciplines required
 - Rehabilitation potential with expectation for clinical and functional improvement ⁽⁴⁵⁾
 - **Skilled nursing service $\geq 4h/d$, $\geq$ One:** ⁽¹¹³⁾
 - Respiratory intervention $\geq 3x/24h$ daily
 - IV medication
 - Nursing intervention or assessment $\geq 3x/24h$
 - Pain management or analgesic ⁽⁵⁹⁾
 - Ventilator management
- **Complete prior to facility transfer, All:**
 - Comprehensive written discharge and teaching instructions reviewed ⁽¹⁰⁴⁾
 - Medication reconciliation ⁽¹⁰⁵⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **Other ALOC** ^(114, 115)

Utilization Benchmarks

Condition or Procedure	LOS (days)	Type
CARDIAC	11	InterQual
GULLIAN BARRE	14.4	InterQual
MAJOR MULTIPLE TRAUMA, (WITH BRAIN INJURY OR SPINAL CORD INJURY)	13	InterQual
MAJOR MULTIPLE TRAUMA, (WITHOUT BRAIN INJURY OR SPINAL CORD INJURY)	12.2	InterQual
NEUROLOGICAL	12.7	InterQual
PULMONARY	9.8	InterQual

Notes:**1:**

Inpatient rehabilitation facility physician program requirements:

- A high level of supervision by a rehabilitation physician and medical management is required to manage conditions related to the admitting diagnosis and comorbidities that may impact the ability to participate in intensive therapies
- Rehabilitation physician provides admission orders, assessment, oversight, and program coordination
- Rehabilitation physician visits face-to-face with the patient at least 3 times per week
- A non-physician rehabilitation practitioner may conduct 1 of the 3 required face-to-face visits beginning the second week of acute rehabilitation stay
- Rehabilitation physician assesses the patient (medically and functionally) and modifies the plan of care as needed
- Interdisciplinary team meeting at least weekly, inclusive of ongoing comprehensive discharge planning

2: Medicare Note

CMS: By selecting this criterion, the reviewer should not only take the patient's medically intensive condition into consideration, but also that the patient must require and can benefit from the intensive rehabilitation program that is unique to an acute inpatient rehabilitation facility. Examples of conditions appropriate for this subset may include, but are not limited to (Centers for Medicare & Medicaid Services. Title 42; Part 412; Subpart B-Classification criteria for payment under the inpatient rehabilitation facility prospective payment system):

- Congenital deformity
- Major multiple trauma
- Neurological disorders (e.g., multiple sclerosis, motor neuron diseases, polyneuropathy, muscular dystrophy, Parkinson's disease)
- Systemic vasculidities with joint inflammation, resulting in significant functional impairment of ambulation or activities of daily living for which appropriate, aggressive, and sustained therapy at a lower level of care has failed, immediately preceding the inpatient rehabilitation admission, but have the potential to improve with an intensive rehabilitation program
- Systemic vasculidities with joint inflammation that result from a systemic disease activation immediately before the inpatient rehabilitation admission, but have the potential to improve with more intensive rehabilitation

3:

Instruction: Selection of this criteria point is also appropriate for non-Medicare beneficiary plans that follow Medicare guidelines.

4:

Instruction: The 30-day time frame begins from the onset of any of the following: illness, injury or exacerbation, the day of surgery, the day treatment or revised treatment is begun, or discharge from acute care for the condition which was treated. When the patient meets the severity of illness criteria, direct admission to this level of care may occur from a variety of settings including the medical practitioner's office, emergency room, urgent care center, or acute care setting.

5:

Transition Plan: InterQual's® Transition Plan identifies patients at high risk for readmission who may benefit from a comprehensive discharge plan. Heart failure (HF) ranks in the top 20 diagnoses with the highest 7-day and 30-day readmission rates. The 30-day all-cause readmission rate is as high as 23%(0.23) (Fingar et al., In: Healthcare Cost and Utilization Project (HCUP) Statistical Briefs. 2017). HF is a common condition in skilled nursing facilities (SNFs), with cardiovascular diagnoses as the largest diagnostic category for this setting. HF patients account for up to 27% (0.27) to 43%(0.43) of SNF 30-day rehospitalization rates (Jurgens et al., J Card Fail 2015, 21: 263-99). HF patients, in general, identified to be at highest risk for readmission include those with:

- Moderate to severe HF and age 65 or older (Iyngkaran et al., Clinical Medicine Insights: Cardiology 2018, 12: 1179546818809358)
- Age less than 65 at the time of initial HF admission (Iyngkaran et al., Clinical Medicine Insights: Cardiology 2018, 12: 1179546818809358)

- Age 65 or older and post major noncardiac surgery (Turrentine et al., J Am Coll Surg 2016, 222: 1220-9)
- Patients with preexisting atrial fibrillation (Tripathi et al., J Am Heart Assoc 2019, 8: e013026)
- Existence of a comorbid condition including diabetes, renal failure, chronic pulmonary disease, anemia, depression, and fluid and electrolyte disorder (Chamberlain et al., Int J Gen Med 2018, 11: 127-41; Arora et al., Am J Cardiol 2017, 120: 616-24)
- High pre-discharge B-type natriuretic peptide (BNP) level and less than 50%(0.50) decrease from admission level
- Poor comprehension of discharge instruction related to limited educational background or primary language other than English (Iyngkaran et al., Clinical Medicine Insights: Cardiology 2018, 12: 1179546818809358)
- Poor medication adherence (Ruppar et al., J Am Heart Assoc 2016, 5:)
- Medicaid coverage (Iyngkaran et al., Clinical Medicine Insights: Cardiology 2018, 12: 1179546818809358)
- Longer initial hospital stays (Arora et al., Am J Cardiol 2017, 120: 616-24; Albert et al., Circ Heart Fail 2015, 8: 384-409)
- Multiple emergency department visits within 6 months of hospitalization (Albert et al., Circ Heart Fail 2015, 8: 384-409)

A BNP level of greater than 350 pg/mL or less than a 50%(0.50) reduction in N-terminal prohormone BNP (NT-proBNP) during the hospital stay is also associated with an increased risk for rehospitalization or death, as is the development of hypotension during hospitalization (Patel et al., Circ Heart Fail 2014, 7: 918-25). Patients with a discharge BNP $\geq$ 1000 pg/mL had an unadjusted 30-day HF-specific readmission rate over 3 times as high as patients whose discharge BNP was $\leq$ 200 pg/mL (Flint et al., J Am Heart Assoc 2014, 3: e000806).

6:

The majority of cardiac rehabilitation occurs in the outpatient setting, although there is a subset of high-risk cardiac patients with comorbid conditions who may require a highly supervised inpatient program. Examples include: status post acute myocardial infarction (MI) or chronic stable angina (CSA), or patients who, during their acute hospitalization, have undergone coronary artery bypass graft (CABG) surgery, cardiac valve surgery, or cardiac transplantation. Comorbid conditions include: status post stroke with residual cognitive and motor impairment, status post amputation, neuromuscular disorder or patients who have developed a medical, neurological, or musculoskeletal complication with physical or functional limitations. The rehabilitation program may consist of graded exercise testing with frequent vital sign monitoring, telemetry, and medication management. Upon discharge from acute and post acute inpatient settings, the American College of Cardiology (ACC) and the American Heart Association (AHA) recommend these patients be referred for outpatient cardiac rehabilitation. Evidence exists that supports cardiac rehabilitation as reducing morbidity and mortality in patients with a recent coronary event (Thomas et al., J Am Coll Cardiol 2018, 71: 1814-37; O'Gara et al., Circulation 2013, 127: e362-425).

7:

Functional limitation refers to the patient who is unable to change position or has difficulty turning side to side, moving from supine to seated position, or raising his or her hips off the bed to distribute pressure and periodically expose the skin to air. These patients are at high risk for wound deterioration and/or the development of additional ulcers over bony prominences and skin maceration from heat occlusion.

8:

Transition Plan: InterQual® Transition Plan identifies patients at high risk for readmission who may benefit from a comprehensive discharge plan. Chronic obstructive pulmonary disease (COPD) patients at high risk for readmission include those with a prior history of exacerbation of COPD requiring hospital admission. Prior emergency room visits and hospitalized exacerbations in the past 12 months are strong predictors for future admissions. Evidence also demonstrates a marked increase in the risk of readmission with each new exacerbation requiring hospitalization (Global Initiative for Chronic Obstructive Lung Disease (GOLD). Global Strategy for the Diagnosis, Management and Prevention of COPD. 2024; Halpin et al., Int J Chron Obstruct Pulmon Dis 2017, 12: 2891-908). Readmission rates have been found to be high among patients of African American race, males, 65 years of age or less, lower-income households, and those with multiple comorbidities. The following conditions are also associated with increased risk of early readmission: congestive heart failure (CHF), chronic renal insufficiency, diabetes, psychiatric conditions (anxiety, depression, psychosis, alcohol and illicit drug use), frailty, poor health-related quality of life, low body mass index (BMI), lack of routine physical activity, oral corticosteroid use, long-term oxygen therapy, elevated serum arterial blood carbon dioxide level, and forced expiratory volume in 1 second

(FEV₁) values below 50%(0.50) of predicted value (Lau et al., Int J Chron Obstruct Pulmon Dis 2017, 12: 1891-902; Tsui et al., Int J Tuberc Lung Dis 2016, 20: 396-401). Interventions that may reduce early readmissions after COPD exacerbation include inhaler device training, early physician outpatient follow-up within 30 days after hospitalization, and self-management interventions that include a COPD exacerbation action plan (Lenferink et al., Cochrane Database Syst Rev 2017, 8: CD011682; Ospina et al., Thorax 2017, 72: 31-9). Additional recommendations to reduce early readmissions include influenza and pneumococcal vaccination, pulmonary rehabilitation program that is at least 6-8 weeks in duration for COPD exacerbation, and access to education and case management that includes direct access to a healthcare specialist at least monthly (Global Initiative for Chronic Obstructive Lung Disease (GOLD). Global Strategy for the Diagnosis, Management and Prevention of COPD. 2024; Halpin et al., Int J Chron Obstruct Pulmon Dis 2017, 12: 2891-908; Criner et al., Chest 2015, 147: 894-942).

9:

Functional impairment refers to any condition that results in activity limitations impacting the patient's ability to ambulate, transfer, and/or perform activities of daily living (ADLs) or instrumental activities of daily living (IADLs). In pediatric patients, functional impairment includes limitations that impact the patient's developmental progress, such as deficits in mental status, sensory functioning, communication, motor functioning, feeding, and respiratory status. The patient will require assistance with one or more limitations.

10:

Spina bifida (SB) is a neural tube birth defect that results in permanent disability and accounts for nearly 90%(0.90) of the total neural tube defects. SB can limit an individual's mobility and cognition. These patients can also have orthopedic deformities such as scoliosis and joint contractures, as well as bowel and bladder dysfunction and psychosocial issues. Renal deterioration related to central nervous system (CNS) damage can lead to end stage renal disease requiring dialysis and eventual renal transplant. SB can limit an individual's mobility and cognition adversely affecting an individual's health-related quality of life. Therapeutic interventions reduce pain and improve physical activity and balance. SB occurs in 7 out of every 10,000 live births in the United States. The prevalence rate 5.04 per 10,000 (1995-1996) has declined 31%(0.31) to a rate of 3.49 per 10,000 (1998-2006) since the U.S. Food and Drug Administration mandated folic acid be added to all enriched cereal grain products (Centers for Disease Control and Prevention, Spina Bifida. 2015; Ouyang et al., J Urol. 2015, 193: 558-64. doi: 10.1016/j.juro.2014.08.092. Epub Aug 25.; Wagner et al., Disabil Health J 2015, 4: 00048-5; Rofail et al., Eur J Obstet Gynecol Reprod Biol. 2014, 181:214-22.: 10.1016/j.ejogrb.2014.07.048. Epub Aug 7.; Crytzer et al., PM R 2013, 5: 1051-62).

11:

Cerebral palsy (CP) is a nonprogressive group of disorders caused by abnormal brain development or injury to the brain during development which affects mobility, balance, and posture. The severity ranges from partial motor impairment to affecting the whole body. CP occurs in about two per 1000 live births and is becoming more prevalent in the adult population due to the increased survival of low birth weight infants. Adults with chronic conditions such as CP are less likely to seek medical care, which may contribute to declining health and need for rehabilitation (Tan et al., Arch Phys Med Rehabil. 2014, 95: 2029-39. j.apmr.2014.04.031. Epub Jun 11.).

12:

End-stage disease is an irreversible, chronic state from which the patient is incapable of recovery.

13:

Instruction: Secondary review is indicated for admission of end-stage malignant disease when the patient has a life expectancy of less than three weeks requiring care and comfort measures only. In determining whether or not the malignant disease is end-stage, consider current treatment being rendered.

14:

The highest incidence of bone metastasis occurs with osteophilic tumors including breast, prostate, lung, kidney, and thyroid. Pain associated with bone metastasis is caused by localized inflammation secondary to periosteal involvement and/or mechanical pain from loss of structural integrity. Inpatient rehabilitation following surgical repair of a pathologic or impending fracture is appropriate. The goal of inpatient rehabilitation is to increase mobility and quality of life. The Karnofsky Performance Index (cancer specific scale), FIM™, the Eastern Cooperative Oncology Group Scale (ECOG) Functional Index, Brief Pain Inventory (cancer specific), and the

Palliative Performance Scale (PPS) are functional assessment tools that can be used to document the effect of treatment on functional capability and outcome (Padro-Guzman and Knowlton, Rehabilitation interventions for metastatic bone tumors. 2017; Saotome et al., Support Care Cancer 2015, 5: 5).

15:

An individualized rehabilitation plan for the patient with cerebral neoplasm must have a multidisciplinary team approach. Therapies should address the effects of the neoplasm upon speech, cognition, vision, strength, balance, coordination, and/or gait. The Karnofsky Performance Index (cancer specific scale), FIM™, or the Eastern Cooperative Oncology Group Scale are examples of functional assessment tools that may be used to document the effect of treatment on functional capability and outcome (Shenouda, PM&R Knowledge Now, American Academy of Physical Medicine and Rehabilitation 2013 Updated Sep 2015 [cited Nov 6 2019]; Khan et al., J Rehabil Med. 2014, 46: 754-60.).

16:

A patient with a diagnosis of malignant or metastatic lung disease must have a functional limitation that is directly impacting their physical status (e.g., exhibits difficulty with ambulation, transfers, activities of daily living, etc.).

17:

Spinal tumors may go unnoticed until neurologic structures are affected. Symptoms include pain that may be accompanied by neurologic deficits, including sensory changes, balance difficulties, weakness, bowel or bladder dysfunction and spasticity. The level of the spinal cord lesion will determine the neurologic or functional deficits. The Functional Independence Measure (FIM™) is one method of tracking functional gains. Inpatient rehabilitation may address medical issues, enhance independence, and provide family education to facilitate the patient's return home (Tommy C. Yu, PM&R Knowledge Now, American Academy of Physical Medicine and Rehabilitation 2013. Updated Aug 2016 [cited Nov 6 2019]; Fortin et al., J Spinal Cord Med 2015, 23: 23; Raj and Lofton, J Spinal Cord Med. 2013, 36: 4-11.).

18:

Trauma refers to a patient who has experienced serious bodily injury to tissues and/or bones as a result of a motor vehicle accident, drowning, falls, burns, assaults, etc. These patients often experience blood loss, organ and bone damage, oxygen deprivation, or shock. A comprehensive rehabilitation program may be required to address new functional impairments and associated activity limitations.

19:

Major trauma includes any severe or critical type of bodily injury that requires timely diagnosis and treatment by a multidisciplinary team.

20:

Neurodegeneration refers to the loss or malfunctioning of brain and spinal cord peripheral nerves, which is characterized by progressive degeneration of the nervous system. Neurodegenerative diseases include: Multiple sclerosis (MS), Parkinson's disease, amyotrophic lateral sclerosis (ALS), and Huntington's chorea. These diseases are usually diagnosed in an ambulatory setting due to the gradual onset of symptoms and may progress steadily or present clinically as an acute exacerbation, worsening of physical function, or a combination of both. Transverse myelitis is also characterized by neurodegeneration, but it is considered to be a nonprogressive condition with sudden onset (24 hours to 2 weeks) of sensory loss and motor weakness ranging from mild weakness to total paralysis. For patients with these conditions, the appropriate level of care is determined by an assessment of the severity of the patient's current condition, the probability of prolonged impairment, or functional limitation, and projected treatment goals.

21:

Instruction: Patient's with progressive disorders (e.g., Parkinson's disease, multiple sclerosis, amyotrophic lateral sclerosis (ALS), etc.) may be appropriate for an inpatient rehabilitation level of care when there has been a sudden and significant decline in the patient's functional impairment status and the potential for significant improvement or recovery exists. Additionally, inpatient acute rehabilitation would be appropriate if frequent adjustments or monitoring of the patient's care is required and cannot be optimally managed in a lower level of care. An admission

for subacute rehabilitation or therapy is appropriate when there has been a gradual decline in status or the patient is at the later stage of their illness (Mallio et al., Neural Regen Res. 2015, 10: 702-3.).

22:

Parkinson's disease (PD) is a progressive neurodegenerative disorder which affects gait, transfers, balance, and posture. Rehabilitation, in addition to traditional medical management, can maximize functional ability and increase independence (Marciniak et al., Idiopathic Parkinson Disease. 2015 [cited Nov 7 2019]).

23:

Myopathy is a neuromuscular disorder of the skeletal muscles, whose symptoms including muscle weakness, cramps, stiffness, and spasm can impact functional activities of daily living (ADL). Myopathy can be inherited (e.g., muscular dystrophy) or acquired. Acquired myopathy may be due to electrolyte imbalance, hormones, infection, inflammation, prolonged illness, or toxins. While inpatient rehabilitation for myopathy may improve ADLs, range of motion and strength, there is limited evidence to demonstrate its effectiveness (Mehrholtz J, Physical rehabilitation for critical illness myopathy and neuropathy (Review). 2015).

24:

Systemic vasculitis is a multisystem disorder characterized by the inflammation of blood vessels and tissue necrosis. Considered to be primary when there is no underlying etiology, vasculitis is secondary if associated with infection (e.g., human immunodeficiency virus, hepatitis B or C) or an underlying connective tissue disease (e.g., systemic lupus erythematosus or rheumatoid arthritis). Morbidity is related to disease complications and immunosuppressive treatment. The patient's ability to perform routine daily activities and to maintain employment are frequently impaired (Lin, Physical Medicine and Rehabilitation - International, 2015, 2).

25:

Acute inpatient pain management programs are designed for patients with pain that impacts their functional ability for managing personal activities of daily living (ADLs). This may require the patient to make significant to substantial modifications to improve functional performance. Patients appropriate for an inpatient rehabilitation program should have failed a trial of at least two outpatient programs consisting of physical or occupational therapy, pharmacologic interventions or trials, and psychological management.

26:

Nonpharmacologic treatment may include transcutaneous nerve stimulation (TENS), massage, acupuncture, application of heat or cold, ultrasound, immobilization, therapeutic exercise, and/or biofeedback.

27:

Impairments of respiration, swallowing, communication, or bowel or bladder function are not appropriate for inpatient therapy unless they occur with a qualifying physical or cognitive activity limitation and the need for skilled nursing services.

28:

Functional assistance levels are based upon the patient's function during tasks and activities necessary to return to household mobility or ambulation. The functional assistance level required for each individual task or activity (e.g., mobility, activities of daily living (ADLs)) may vary.

The following terms are commonly used in the post-acute setting:

- Independent - Patient can safely and within a reasonable amount of time perform a task (or developmentally appropriate task) without physical or cognitive assistance or supervision
- Modified Independent - Patient performs an activity with a supportive device, adaptive equipment, and/or prosthetic or orthotic device. Additional time may be required to complete the activity and/or there are safety (risk) considerations
- Supervision - Patient performs an activity with standby or distant supervision or setup. Verbal cueing or coaxing, without physical contact or setup of items and application of orthoses may be required when patient's safety awareness is impaired
- Minimum or limited Assistance - Patient performs at least 75%(0.75) of an activity and requires some physical contact to steady, guide, or move

- Moderate or extensive Assistance - Patient performs at least 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Maximum Assistance - Patient performs 25%(0.25) to 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Total Assistance or dependence - Patient performs less than 25%(0.25) of an activity and may require total assistance for functional mobility or ADLs

29:

Activities of daily living (ADLs) are fundamental skills required to independently care for oneself. Examples of ADLs include, but are not limited to:

- Bathing
- Dressing
- Eating
- Toileting

30:

Cognition includes short- and long-term memory, problem solving, and executive functioning (e.g., awareness or insight with respect to deficits, error detection, impulse control).

31:

Dysarthrias are a group of motor speech disorders characterized by slow, weak, imprecise, or uncoordinated movements of speech musculature. Word retrieval and comprehension of verbal and written language are preserved but articulation is impaired.

32:

Functional mobility refers to a patient's ability to move independently. Examples of functional mobility include, but are not limited to:

- Bed mobility
- Sit to stand
- Stair negotiation
- Transfers
- Walking
- Wheelchair mobility

33:

Respiratory impairment may include anatomic or physiologic alteration that interferes with systemic response to oxygen demands (e.g., airway clearance, respiration, ventilation).

34:

The criteria "at least minimum assistance" refers to patients who require minimum, moderate, maximum, or total assistance.

35:

The intensity of rehabilitation therapies is a driver for admission to an acute inpatient rehabilitation facility. Documentation should reflect the need for required and ongoing intensive therapies. The determination of whether a patient requires and can tolerate an intensive rehabilitation program of at least 15 hours is not based on any one single piece of documentation. All clinical documentation should be evaluated by the reviewer to determine whether the patient is demonstrating the need for services and has the ability to tolerate them. Patients who do not require this level of intensive therapy should be treated at a lower level of care. Rehabilitation therapies should be reasonable and medically appropriate and should not exceed a patient's level of tolerance. If there are clinical features which preclude the patient's ability to tolerate the intensity threshold of at least 15 hours per week, secondary review would be appropriate (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Miller and Forer, Pm r 2021, 13: 535-9; Forrest et al., Medicine (Baltimore) 2019, 98: e17096).

36: Medicare Note

CMS: Medical appropriateness for the inpatient rehabilitation facility admission is determined by the need for an interdisciplinary team approach. Documentation should demonstrate the requirement for complex nursing, medical management, and rehabilitation care. Comprehensive and interdisciplinary rehabilitation services coordinated by a rehabilitation physician must include rehabilitation nursing, social work and/or case management, and at least 2 therapy disciplines (1 of which must be physical or occupational therapy). Additional services may include speech therapy, prosthetic-orthotic services, psychology, neuropsychology, respiratory therapy, nutritional counseling, therapeutic recreation, and/or vocational counseling (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

37:

Treatment precluded in a lower level of care due to clinical complexity includes the following:

- Medical practitioner assessment or intervention at least 3 times per week
- Specialized therapeutic skills or equipment required
- Rehabilitation nursing services available 24 hours per day

38: Medicare Note

CMS: According to the Centers for Medicare & Medicaid Services, intensive inpatient rehabilitation therapy services consist of at least 3 hours of therapy per day and at least 5 days per week as a general "rule of thumb." However, in well documented cases, the intensity of therapy services can be delivered in various increments provided that the patient meets the requirement of at least 15 hours of skilled therapy per week (i.e., 7 consecutive calendar day period) (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

39: Medicare Note

CMS: Services are considered skilled when they are ordered by a physician and when they are so inherently complex that they can only be performed safely and effectively by, or under the supervision of rehabilitation therapists. The therapy plan of care must be documented and is considered "established" once written or dictated by the rehabilitation therapist. Therapy services must be directly related to the plan of care. The plan of care also generally includes details of the anticipated therapy interventions (i.e., expected intensity, frequency, duration). Treatment goals should be documented throughout the patient's admission, measurable, and directly related to the patient's functional impairments (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 15 - Covered Medical and Other Health Services. 2021).

40: Medicare Note

CMS: For a patient to benefit from care provided at an inpatient rehabilitation facility, their clinical stability must support an intensive level of skilled therapies. In the case where a patient's clinical stability precludes them from tolerating intensive and ongoing therapies or where the patient has not yet completed a full course of acute hospital treatment, the patient should remain at a higher level service intensity (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

41:

Patients should be clinically stable within the past 24 hours and not require acute medical attention.

42:

Full participation requires the patient to be medically stable, cognitively capable, and willing to participate in an intensive therapy program. The expected result is that the patient will show a significant therapeutic improvement over a clearly documented period of time.

43: Medicare Note

CMS: The preadmission screen serves as documentation of the patient's prior level of function (prior to current

condition), condition requiring acute rehabilitation, expected progress, expected length of stay, risk of complications, treatments/therapies required, and expected discharge destination. The preadmission screen should be completed by a licensed or certified clinician(s) within 48 hours immediately preceding admission to the inpatient rehabilitation facility (IRF). In certain circumstances, when the preadmission screening is completed more than 48 hours prior to the patient's admission to the IRF, an update to the preadmission screening must be completed (in person or by telephone) that reflects the patient's medical and functional status and documented in the IRF's medical record within 48 hours preceding the patient's admission to the IRF. The clinician(s) are qualified by education, professional licensure, or specialized training and experience in rehabilitation (e.g., physical therapist, occupational therapist, rehabilitation nurse). A rehabilitation physician is responsible for reviewing, concurring, and signing the preadmission screening (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

44: Medicare Note

CMS: While admitted in the inpatient rehabilitation facility, documentation is required to demonstrate expected measurable improvement within a prescribed period and goals are of practical value to the patient's functional status or adaptation to impairments. The patient's progress and improvements are expected to be ongoing and sustainable. The patient is not required to achieve their prior level of function or be completely independent with self-care to benefit from acute inpatient rehabilitation (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

45:

Rehabilitation potential refers to the probability that therapy and medical goals are realistic and attainable based on patient's prior level of function, severity of illness or injury, and the extent of impairments.

46: Medicare Note

CMS: Medicare beneficiaries admitted to an inpatient rehabilitation facility must receive services from at least 2 therapy disciplines, 1 of which must be physical therapy or occupational therapy (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

47:

Inhalation injuries may affect the upper airway as a thermal insult, or lung parenchyma secondary to inhalation of noxious substances such as smoke. A patient may experience a combination of both a thermal insult and inhalation injury. Burns to the face, anterior chest wall, or any accessory breathing muscles may result in restrictive lung disease, pulmonary edema, atelectasis, or pneumonia. Smoke inhalation may increase mortality by 20%(0.20). When combined with pneumonia it can increase mortality up to 60%(0.60) (Asmussen et al., Burns 2013, 39: 429-35).

48:

A certified orthotist and/or prosthetist provides comprehensive care related to the creation of artificial limbs (protheses), orthotics, and braces. The prosthetics/orthotics service must be carried out by a certified prosthetist and/or orthotist.

49:

The individualized plan must include time limited goals, expectations, and input from the patient and/or family whenever possible, projected functional outcomes, and an expected discharge plan, inclusive of a discharge destination (American Physical Therapy Association, Guidelines: Physical Therapy Documentation of Patient/Client Management. 2014 [cited Nov 7 2019]). The plan of care is developed jointly by the interdisciplinary team (e.g., OT, ST, PT, rehabilitation nursing, and the rehabilitation medical practitioner).

50: Medicare Note

CMS: The patient's need for inpatient rehabilitation admission is supported by an individualized plan of care. The plan of care should generally include (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual

Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021):

- Medical prognosis
- Anticipated interventions
- Functional outcomes
- Discharge destination

51: Medicare Note

CMS: Treatment goals include the anticipated goals and expected functional outcomes used for establishing a treatment plan. They are specific, measurable, and achievable objectives to address areas of impairment and activity limitations such as activities of daily living and mobility. Required rehabilitative therapies (at least 2 disciplines, 1 of which must be physical therapy or occupational therapy) must begin within 36 hours from midnight of the day of admission to the facility. Therapy evaluations are generally accepted as the initiation of required therapy (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

52:

These criteria refer to medical (e.g., bowel or bladder, respiratory, nutrition, wound or skin, pain, medications) and psychosocial issues (e.g., coping skills, adjustment to functional loss) that may impede functional progress and require active treatment, ongoing assessment, and management during the course of inpatient rehabilitation facility stay.

53:

Bowel management includes the assessment of bowel dysfunction (e.g., evaluation of bowel function pattern, rectal muscle weakness, hydration, and nutritional status) and the implementation of an individualized treatment and/or training program with individuals and their families. The bowel regimen may include diet modification, exercise training, medications, scheduled stooling (e.g., with rectal stimulation or suppository for neurogenic bowel), or mechanical disimpaction. Patients and their families must be educated in following the prescribed treatment plan, as well as recognizing and reporting signs of difficulty (e.g., urgency, dysuria, diarrhea, or constipation) (Winstein et al., Stroke 2016, 47: e98-e169).

54:

Assessing premorbid bladder and bowel patterns is important before interventions are performed. Bladder emptying as well as urinary incontinence may be monitored with individuals that have neurogenic bladder, urinary retention, or hyperreflexia with urge incontinence (Winstein et al., Stroke 2016, 47: e98-e169). Bladder management includes the assessment of bladder dysfunction and the implementation of an individualized treatment and/or training program. The program may include the establishment of a voiding schedule (timed voiding), urine collection devices (e.g., external condom catheters, indwelling catheter, intermittent self-catheterization), behavioral management techniques (e.g., bladder stimulation, Valsalva and Crede maneuvers, pelvic floor exercises), and medication management (e.g., anticholinergic agents, muscle relaxants, etc.).

55: Medicare Note

CMS: The Centers for Medicare & Medicaid Services has identified inpatient hospital-onset *Clostridioides difficile* infection as a quality data collection point for inpatient rehabilitation facilities (Centers for Medicare & Medicaid Services, Inpatient Rehabilitation Facilities (IRF) quality reporting program. 2019).

56: Medicare Note

CMS: The Centers for Medicare & Medicaid Services has identified catheter-associated urinary tract infections (CAUTI) as a quality data collection point for inpatient rehabilitation facilities (Centers for Medicare & Medicaid Services, Inpatient Rehabilitation Facilities (IRF) quality reporting program. 2019).

57:

Patients having difficulty coping with their illness or adjustment to functional loss may exhibit denial, anger, unrealistic expectations or nonadherence with their treatment plan, etc. Skilled services may be required to address these behaviors.

58:

Medication management includes an assessment of the patient's tolerance, the effectiveness of the prescribed medications, the patient's adherence with the regimen, and adverse effects of the medications.

59:

Pain management includes assessment and interventions to relieve pain or decrease its intensity and its effects on lifestyle (e.g., strategies to reduce perception of pain, use of pharmacologic and/or non-pharmacologic pain control modalities, psychosocial counseling, and recommendations for modifications in work, school, or leisure activity). Non-pharmacologic pain interventions may include progressive muscle relaxation, massage, guided imagery, positive visualization, or music therapy (American Academy of Pain Medicine, Chronic pain medical treatment guidelines. 2009 [Cited Oct 9, 2019]).

60:

Untreated spasticity can lead to contracture and activity limitations ranging from difficulties cleaning hands to difficulty ambulating. It should be treated if it causes pain or affects mobility. Targeted injections of botulinum toxin into upper limb muscles reduce spasticity, improve range of motion (ROM) or active ROM, improve dressing, hygiene, and limb positioning; injections into lower limb muscles may also reduce severe hypertonicity in hemiplegic shoulder muscles. Neuromodulating pain medication may also help, as well as ROM and repositioning (Winstein et al., Stroke 2016, 47: e98-e169).

61:

Acute changes in chronic wounds may warrant complex nursing care at a skilled nursing facility (SNF) or subacute care (SAC) facility. Wound or skin care involves daily observation and assessment of a patient's altered skin integrity, including burns and grafts. Interventions that may be necessary include:

- Wound debridement
- Soaks and irrigations
- Whirlpool
- Specialized occlusive dressings (excluding dry sterile dressing changes)
- Packings
- Specialized treatments
- Pressure redistribution beds
- Patient and/or caregiver education

62: Medicare Note

CMS: The Centers for Medicare & Medicaid Services has identified changes in skin integrity such as pressure ulcer/injury as a quality data collection point for inpatient rehabilitation facilities (Centers for Medicare & Medicaid Services, Inpatient Rehabilitation Facilities (IRF) quality reporting program. 2019).

63:

Discharge planning is a component of care coordination and should be initiated on admission and re-evaluated throughout the patient's stay. The discharge plan may include an estimated length of stay, projected discharge destination, and a plan for post-discharge care. It may also include the following:

- Durable medical equipment needs
- Home environment assessment
- Identification of community resources
- Patient and/or caregiver education and instruction
- Patient's support system assessed, and level of care options identified

64: Medicare Note

CMS: Discharge planning begins upon a patient's admission to an inpatient rehabilitation facility. The objective is for the patient to be able to safely return home or to a community-based environment. The interdisciplinary team, which includes a rehabilitation physician, a registered nurse, a social worker and/or case manager, and a rehabilitation therapist from each discipline involved in the patient's care, is responsible for developing and following through with the care coordination for the patient. Regular team conferences (at least once per week) should address (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient

Hospital Services Covered Under Part A. 2021):

- Assessment of the patient's progress toward established goals
- Addressing barriers to the patient's progress toward goals
- Reassessment of established goals, as needed
- Revising the plan of care, as needed

65:

Selection of this criteria point indicates that the patient is responding to treatment and is clinically stable for transfer or discharge. To determine the most appropriate post-acute level of care, see discharge criteria.

66:

Neurological stability is a clinical state characterized by:

- Lack of deterioration or return to baseline in mental status or level of consciousness
- Seizures absent or controlled
- Absence of neurological deficit
- Stabilization of neurological deficit that develops during current hospitalization

67:

A patient who is medically stable maintains vital signs within their individual baseline, and sustains a stable pulmonary and hemodynamic status.

68:

Pain is considered to be adequately controlled when a stable medication regimen and/or adjunct therapy provides sufficient, symptomatic relief without causing significant adverse side effects. Studies have shown epidural analgesia to be very effective in the initial post-operative period (McNicol et al., Cochrane Database Syst Rev 2015: CD003348; Popping et al., Ann Surg 2014, 259: 1056-67). For acute pain, the Center for Disease Control (CDC) recommends prescribing the lowest effective dose of immediate-release opioids, with careful attention paid to the quantity and duration of the prescription. Three days or less will often work; more than 7 days is rare. For chronic pain unrelated to active cancer, palliative or end-of-life care, the CDC recommends the following guidelines for clinicians (Dowell et al., Jama 2016, 315: 1624-45; Dowell et al., MMWR Recomm Rep 2016, 65: 1-49):

- Consider nonpharmacologic therapy and nonopioid pharmacologic therapy first
- Know the patient's history of controlled substance prescriptions, using the state prescription drug monitoring program (DMP) data when initiating opioid therapy and then every 3 months thereafter on every prescription
- Weigh the risks and benefits of opioid therapy before starting or continuing opioids
- Establish realistic treatment goals, discuss risks and benefits of opioid therapy, and go over the plan for discontinuing opioids if the harm outweighs the benefits
- Discuss patient and clinician responsibilities for managing this therapy. If a patient has an opioid use disorder, offer evidence-based, medication-assisted treatment in combination with behavioral therapies

When prescribing opioids for chronic pain:

- Combine them with nonpharmacologic therapy/nonopioid therapy
- Prescribe immediate-release opioids versus extended-release or long-acting opioids
- Start at the lowest dose
- Evaluate the benefits and the harm with patients within 1 to 4 weeks of starting opioid therapy, and then at least every 3 months thereafter
- Consider annual urine testing to assess for prescribed medications as well as other controlled prescription and/or illicit drugs
- Avoid prescribing opioid pain medication and benzodiazepines concurrently

For patients in the early post-surgical phase, a large retrospective cohort study found that the duration of the opioid prescription, rather than its dosage, was more strongly associated with opioid misuse (Brat et al., BMJ 2018, 360: j5790).

69:

Prior level of function refers to the patient's level of function prior to the onset of this episode of illness or injury.

This must be taken into consideration when discharge goals are identified.

70:

The ability to ambulate 50 feet in a reasonable amount of time satisfies the definition for household distance.

71:

Functional plateau refers to the situation in which a patient shows no significant functional improvement in a reasonable amount of time. This is exhibited when the patient continues to require the same level of assistance following training by professional staff, and/or the percentage of measurable progress is unchanged or inconsistent. Minimal functional gain is demonstrated when there has been little or slow functional improvement, but it is expected that continued therapy will result in further functional gains. For example, a patient with a head injury and neurological deficits may demonstrate minimal functional gains, but may benefit from rehabilitation over a longer period of time, at a less intense level of care.

72: Medicare Note

CMS: For patients to qualify for continued stay at an inpatient rehabilitation facility, they must demonstrate the need for ongoing intensive inpatient rehabilitation and require the services of an interdisciplinary team (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

73:

Documentation of measurable progress is required in order to justify the need for continued skilled therapy. This entails an evaluation of the patient's progress toward the documented goals in both structural and/or functional impairments. Specific measures should include range of motion, strength, functional status, the level of assistance required, pain level, posture, balance, gait, etc. Progress in physical function is measured against the prior assessment (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

74:

Those patients who have sustained a burn and who are participating in an intensive inpatient therapy program should show continued, measurable improvement. Improvement can be evaluated by assessing whether the patient has a decreased incidence of contracture, an increased range of motion, increased strength and endurance, improved transfer and mobility capabilities and an improved competency with the use of adaptive equipment.

75:

Improvement in activities of daily living (ADLs) can be measured by an increased independence in areas such as bathing, grooming, and dressing. The goal is steady progression towards performing these activities with less assistance.

76:

Role function refers to the performance of activities such as employment, housework, and school.

77:

Pulmonary rehabilitation includes techniques to maximize ventilation and aerobic capacity (e.g., energy conservation for activity and speech, breathing re-education, postural drainage, manual therapy techniques [e.g., percussion or vibration], and use of airway clearance techniques [e.g., vest therapy or flutter valves]). Improved pulmonary function can be measured by the patient's ability to tolerate an increase in exercise, activity level, and/or enhanced ability to perform ADLs, and/or energy conservation techniques. Respiratory symptoms such as dyspnea, sputum production, frequency of cough and wheezing should improve with performance of these activities (Rochester et al., Am J Respir Crit Care Med 2015, 192: 1373-86; Ries et al., Chest 2007, 131: 4S-42S).

78:

The most common forms of endurance exercise in a pulmonary rehabilitation program are walking and cycling, but may also include stair climbing, inspiratory muscle training, and arm exercise training. By conditioning the muscles

of ambulation and improving cardiovascular fitness, improvement in physical activity can lead to a reduction in fatigue and breathlessness (Rochester et al., Am J Respir Crit Care Med 2015, 192: 1373-86; Ries et al., Chest 2007, 131: 4S-42S).

79: Medicare Note

CMS: To benefit from an acute inpatient rehabilitation program, there is an expectation that the patient will exhibit a measurable, practical improvement in their functional status within a predetermined and reasonable time frame. The expected length of time that it will take for the patient to achieve improvement should be assessed and agreed upon by the interdisciplinary team and the medical rehabilitation practitioner. This should be documented as part of the patient's treatment plan (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021).

80:

Recommendations to prevent skin breakdown include minimizing or eliminating skin friction, minimizing skin pressure, providing appropriate support surfaces, avoiding excessive moisture, and maintaining adequate nutrition and hydration. Patients should be turned at least every 2 hours, wheelchair seating or cushioning must be appropriate, and appropriate special mattresses should be used until mobility returns (Young et al., Phys Med Rehabil Clin N Am 2019, 30: 111-32; Winstein et al., Stroke 2016, 47: e98-e169).

81:

During inpatient rehabilitation, regular skin assessments are recommended with objective scales of risk (e.g., Braden scale). Pressure ulcers are associated with impaired circulation, older age, and incontinence (Winstein et al., Stroke 2016, 47: e98-e169). Advanced static mattresses or advanced static overlays are recommended for individuals at an increased risk of developing pressure ulcers (Qaseem et al., Ann Intern Med 2015, 162: 359-69).

82:

Instruction: In a 7 day review period, any new onset of medical instability leading to decreased participation in the intensive rehabilitative therapies for more than 3 days, consecutive or non-consecutive, should be referred for secondary review.

83:

Instruction: In the case where a patient cannot tolerate an intensive rehabilitation program for at least 15 hours per week due to a medical instability, application of this criteria point is appropriate for no more than 3 days.

84: Medicare Note

CMS: There must be appropriate documentation specifying the reason as to why the Medicare beneficiary has decreased participation in their intensive rehabilitation program. Decreased participation in their intensive rehabilitation program due to medical instability is not to **exceed 3 consecutive calendar days** (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021). Patients whose medical instability and decreased participation in therapy exceeds 3 days should be referred for secondary review.

85:

Instruction: This line of criteria is appropriate for patients who have a suspected DVT who require bed rest prior to a confirmed DVT diagnosis.

86:

The expected treatments for chronic obstructive pulmonary disease (COPD) include:

- Supplemental oxygen
- Bronchodilators
- Corticosteroids

Some patients may also receive an anti-infective as part of their treatment (Global Initiative for Chronic Obstructive

Lung Disease (GOLD). Global Strategy for the Diagnosis, Management and Prevention of COPD. 2024).

87:

Exercise-induced symptoms may occur with normal therapy. If symptoms produce instability and are sustained after a period of rest, further diagnostic workup may be indicated or a change in treatment may be needed (e.g., new or adjusted medication, oxygen therapy initiation or adjustment).

88:

Oxygen therapy is the administration of oxygen at concentrations greater than ambient air (room air: 21%(0.21)) with the intent of treating and/or preventing the symptoms and manifestations of hypoxia. The oxygen concentration or percentage (FiO_2) delivered varies with the manufacturer's design, oxygen flow rate, the patient's respiratory rate, and tidal volume. Actual inspired FiO_2 with nasal cannula varies significantly with the patient's minute ventilation and pattern of breathing. For patients at risk for CO_2 retention, where a precise inspired FiO_2 is required, the Venturi mask is the preferred method. In general, patients who are very ill or have respiratory disease may require considerably higher flow rates to achieve the desired FiO_2 . The following are estimates of O_2 delivered at the associated flow rates:

LOW-FLOW SYSTEMS**Nasal cannula**

Room air 21%(0.21)

1L 24%(0.24) **4L** 36%(0.36)

2L 28%(0.28) **5L** 40%(0.40)

3L 32%(0.32) **6L** 44%(0.44)

Simple oxygen masks

- 35-50%(0.35-0.50) FiO_2 (5-10 L/min)
- Flow rates are usually maintained at 5 L/min or more to avoid accumulation of CO_2 in the mask

Venturi masks

- 24-50%(0.24-0.50) FiO_2 (4-12 L/min)

Partial rebreathing masks

- 40-70%(0.40-0.70) FiO_2 (6-10 L/min)

Non-rebreathing masks

- 60-80%(0.60-0.80) FiO_2 (minimum flow of 10 L/min)

HIGH-FLOW SYSTEMS**Air-entrainment masks/nebulizers**

- 24-40%(0.24-0.40) FiO_2

Heated, humidified high-flow concentrating nasal cannula (HFNC)

- 24-100%(0.24-1.0) FiO_2

89:

Baseline refers to either the patient's normal baseline or a newly established baseline. In the absence of documentation, a patient's baseline status may be presumed to be normal.

90:

Syncope is the transient loss of consciousness (LOC) and postural tone caused by diminished cerebral blood flow. It is characterized by rapid onset, short duration, and spontaneous complete recovery.

Presyncope is an episode of near-fainting or a sign of impending LOC. Symptoms include, but are not limited to, dizziness, lightheadedness, blurred vision, and general unsteadiness.

91:

Established hemodialysis is generally performed 3 times per week and some fatigue prior to or post-dialysis treatment is expected. Therapy schedules should be adjusted around planned or established invasive procedures or treatments. Unexpected fatigue may occur if the patient is new to the dialysis regimen or if other invasive

procedures are scheduled (e.g., endoscopic retrograde cholangiopancreatography (ERCP), computerized tomography (CT) with contrast or central line change). In these instances, therapies may need to be delayed or reduced accordingly.

92:

Minimally invasive procedures may include arthroscopy, endoscopy, interventional radiology, or interventional cardiology.

93:

A patient experiencing a heart failure (HF) exacerbation may exhibit an increase in their signs or symptoms of heart failure which may include shortness of breath, edema, fatigue, or confusion.

94:

Vital signs are considered sustained findings when they are abnormal for 2 or more readings greater than 15 minutes apart or are lasting at least 15 minutes. This excludes an isolated reading, a transient abnormal measurement, or a finding that requires urgent treatment (e.g., severe hypoxia, ventricular arrhythmia, or hypotension). Application of criteria for a vital sign finding should be based on confirmatory measurements repeated at regular intervals. Events that are infrequent or vital sign abnormalities that have resolved with outpatient treatment are not considered to be sustained (e.g., the resolution of hypertension following anti-hypertensive therapy or the resolution of tachycardia following rehydration).

Note: The definition of sustained reflects the opinion of InterQual's® expert clinical consultants. The criteria are based upon current best practice and are the product of an iterative process involving multiple clinicians with diverse expertise in varied practice and geographic settings.

95:

The Centers for Disease Control and Prevention (CDC) define a fever as greater than or equal to 100.4 degrees Fahrenheit (i.e., greater than or equal to 38 degrees Celsius) and do not specify the route of measurement (Centers for Disease Control and Prevention, Definitions of symptoms for reportable illness. 2017). The route utilized may be directed by institutional protocols, equipment availability, patient preference, or other patient-specific factors.

96:

Mental status change may include confusion, disorientation, delirium, or increasing lethargy (Lacey et al., Ann Med 2019, 51: 232-51).

Instruction: Patients with acute coma, stupor, or obtundation should be reviewed at a higher level of care due to the need for more frequent neurological monitoring. These criteria exclude chronic coma, stupor, or obtundation (Shenvi et al., Ann Emerg Med 2020, 75: 136-45).

97:

A neurological assessment establishes a baseline so that subtle changes can be monitored. A comprehensive neurological assessment often includes an evaluation of:

- Mental status (e.g., level of consciousness, orientation, insight, calculation ability)
- Cranial nerves (e.g., olfactory, optic, vestibulocochlear)
- Motor system (e.g., muscle tone, strength, reflexes)
- Sensory system (e.g., light touch, pain, temperature)
- Coordination (e.g., orchestration and fluidity of movement)
- Gait (e.g., heel-to-toe straight-line walking)

98:

Changes in neurologic status include aphasia, dysarthria, vision or visual field changes, dysphagia, paresthesias, and paralysis.

99:

Pain that interferes with therapy should be monitored, assessed, and documented (pain parameters include ratings of quality, intensity, location, frequency, duration, and exacerbating or alleviating factors) at least daily to

determine the degree of clinical improvement or deterioration. Appropriate medication adjustment(s) and the necessary interventions should be taken to reduce pain so that participation in therapy can be maximized.

100:

The discharge screen is a resource tool and not criteria. Referring to the discharge screen at the initiation of discharge planning is recommended.

101:

The home environment and safety assessment will normally include an evaluation of the patient's pre- and post-hospitalization functional level, physical layout of the home (e.g., entrances and exits, stairs, access to the community), identification of unsafe conditions (e.g., scatter rugs, missing handrails, oxygen use and smoking, lack of fire safety devices, inadequate lighting, heating, and cooling), factors that may trigger symptoms (e.g., secondhand smoke, poor food choices, dysfunctional family dynamics), sanitation hazards (e.g., lack of electricity, running water, refrigeration, inadequate toilet facilities, presence of insects or rodents), and the need for adaptive equipment (e.g., walker, handrails for the tub or shower, elevated toilet seat).

102:

Components of a home therapy regimen may include instruction in strengthening, flexibility, postural retraining, ADL practice, swallowing/communication, use of assistive devices (e.g., braces, gait equipment, application of splints/pressure garments), respiratory compensatory techniques (e.g., purse lip breathing and other respiratory modalities), safety considerations, and how the family and/or caregiver should assist with these program components.

103:

Close follow-up of older adults, pediatric, or at-risk patients after discharge can help minimize hospital readmission and total health care costs (Rising et al., Ann Emerg Med 2013, 62: 145-50; Frei-Jones et al., Pediatr Blood Cancer 2009; 52(4): 481-485; Kripalani et al., JAMA 2007; 297(8): 831-841). Studies have shown that for greater than 50% (0.50) of patients re-hospitalized within 30 days of discharge, there was no visit to a physician's office between the time of discharge and re-hospitalization (Hernandez et al., JAMA 2010, 303: 1716-22; Jencks et al., N Engl J Med 2009; 360(14): 1418-1428). Efforts to reduce readmissions should focus on patients known to be at high risk and patients who have frequent visits to the emergency department (ED). Patients at high risk may have multiple ED visits prior to readmission (Rising et al., Ann Emerg Med 2013, 62: 145-50).

104:

Ensuring the patient and/or caregiver understands all aspects of the condition and can assume responsibility for self-care is crucial in preventing readmission. Assessing a patient and/or caregiver's level of understanding after education has been provided can be difficult. The teach-back method is an effective way of providing education at the appropriate level and can also be used to assess the learner's comprehension. To use the teach-back method, discuss key points in common terms and avoid using medical jargon or unfamiliar terms. After the education has been delivered, ask the learner to repeat what was learned. Gaps or misinterpretations in the learner's explanation will pinpoint areas where communication may have failed and provide opportunity for clarification.

105:

Medication reconciliation is a formal process or technique used by health care providers and pharmacists to identify the most complete and accurate list of all medications a patient is taking at times of transitions in care (e.g., upon hospital admission, transfer from one unit to another during hospitalization, or discharge from the hospital to home or another facility). The goals of this process are to ensure medication and dosages are appropriate for the patient, resolve discrepancies in drug regimens, and ultimately prevent medication errors and reduce adverse drug events. Medication reconciliation is a Joint Commission National Patient Safety goal. Coordinating information when a patient is transferred to another setting, service, practitioner, or level of care ensures accurate medications are listed. The process is comprised of the following (Hospital: National Patient Safety Goals. 2020):

- Obtain an external list of medications (e.g., medications taken prior to admission)
- Develop a list of current medications and add them to the medical record
- Compare the medications from the external list to the current list
- Clarify inconsistencies/discrepancies (e.g., omissions, duplications, contraindications, unclear

information, and changes)

- Develop a list of medications to be prescribed at discharge or transfer
- Communicate the new list to the patient and/or appropriate caregiver(s)
- Ensure that patient and/or caregiver(s) understand the medication information upon discharge

Specific medication issues identified as being problematic include missing medication information from transfer orders, lack of information on medications provided in the acute setting, incomplete medication records, discrepancies between hospital regimen and discharge summary, and missing information pertaining to the patient's tolerance of a medication regimen. Although medication reconciliation is an important aspect in patient safety, there is a lack of consensus and evidence about the best effective methods of implementing this process. Physician-led and electronic medication reconciliation in hospitals are effective strategies to reduce medication discrepancies, however, the impact of these interventions is uncertain due to the low quality of evidence (Choi and Kim, J Clin Pharm Ther 2019, 44: 932-45; Redmond et al., Cochrane Database Syst Rev 2018, 8: CD010791). Trained pharmacy technicians, under the direction of a licensed pharmacist, may be an option for developing and expanding medication reconciliation processes (Irwin et al., Hosp Pharm 2017, 52: 44-53).

106:

The Centers for Medicare & Medicaid Services has identified the transfer of health information to the patient's post acute care (PAC) provider as a quality data collection point for inpatient rehabilitation facilities. This measure collects data on whether a reconciled medication list was provided to the patient's health care provider when the patient was discharged from the PAC setting. Data collection for this quality indicator began October 1, 2020 (Center for Medicare and Medicaid, Inpatient Rehabilitation Facility (IRF) Prospective Payment System for Fiscal Year 2020 and Updates to the IRF Quality Reporting Program. 2019).

107:

The Centers for Medicare & Medicaid Services has identified the transfer of health information to the patient from the post acute care (PAC) facility as a quality data collection point for inpatient rehabilitation facilities. This measure collects data on whether a reconciled medication list was provided to the patient or their caregiver when a patient was discharged from the PAC setting. Data collection for this quality indicator began October 1, 2020 (Center for Medicare and Medicaid, Inpatient Rehabilitation Facility (IRF) Prospective Payment System for Fiscal Year 2020 and Updates to the IRF Quality Reporting Program. 2019).

108:

Completion of family training, with the goal of the patient and/or family being able to safely manage care, may include demonstrating knowledge and aptitude in areas such as transfer skills, medication management, application of splints/pressure garments, skin care, and knowledge of community resources.

109:

The initial clinical assessment is performed by a licensed professional and includes a comprehensive review of the patient's presenting diagnosis and a review of body systems. Identification of current and potential medical needs and health problems can be identified by the clinical assessment. The clinical assessment includes:

- History and physical exam (e.g., vital signs, height, weight)
- Pain assessment includes cause, intensity, quality, onset, duration, and effects on quality of life (e.g., activities of daily living (ADLs), instrumental activities of daily living (IADLs), and interpersonal relationships). Pain relievers should also be included
- Nutritional and hydration status
- Functional ability
- Safety and infection control measures
- Prescribed and over-the-counter (OTC) medications, herbal supplements, and home remedies
- Patient's and/or caregiver's understanding of medication use, dosing, side effects, and adherence to the medication or treatment regimen
- Patient's and/or caregiver's understanding of the illness or disease process and potential long-term complications
- Patient's and/or caregiver's coping strategies to deal with the illness or injury and ability to follow the plan of care

110:

Patient and caregiver education may be provided by one-on-one instruction, audiovisual aids, simulations and role-playing, and self-instruction modules. An assessment of the patient's and/or caregiver's understanding of the material provided may be evaluated by a demonstration of the specific skills or by a verbal response indicating comprehension of the material. Topics for teaching may include disease process, medication administration, indications and effects, diet, prescribed exercises, or treatments (e.g., dressing changes, oxygen administration, blood glucose monitoring, enteral feedings), safety precautions, importance of follow-up with health care provider, emergency management, and community services.

111:

Treatment precluded in a lower level (e.g., home care services, outpatient clinic) is dependent upon several factors including:

- The patient's caregiver or family support system
- The availability of qualified alternate levels of care
- The patient's benefit plan

The patient's medical needs should be met at the least intensive level of care that can safely provide the necessary services.

112:

Skilled nursing services refers to services that must be provided by a licensed nurse who is qualified to assess and monitor patient condition(s) and provide medical treatment and/or teaching to patients who have skilled nursing needs. Examples of Skilled Nursing Facility level of care interventions include:

- Parenteral medications $\geq 2x/day$
- Assessment of clinical status at least daily
- Management and evaluation of a care plan
- New enteral feeding management and teaching
- Patient and/or caregiver education

113:

Skilled nursing services refers to services that must be provided by a licensed nurse who is qualified to assess and monitor patient condition(s) and provide medical treatment and/or teaching to patients who have skilled nursing needs. Examples of Subacute level of care interventions include:

- Multiple intravenous (IV) medications
- Single IV medication given $\geq 3x/day$
- Chronic ventilator management
- Monitoring and assessment of conditions where clinical assessment is required $\geq 3x/24h$ in combination with multiple parenteral medications and/or therapies
- Daily or every other day blood or albumin transfusions
- Multiple and frequent (3-4x/24h) interventions such as respiratory, wound management, and/or total parenteral nutrition (TPN) or peripheral parenteral nutrition (PPN)

114:

Based on the patient's current medical condition and expected discharge needs, other services, or alternate levels of care (ALOC) may be appropriate to adequately address any medical, psychiatric, or substance-related disorder needs.

115:

Day rehabilitation is appropriate for a patient transitioning from an inpatient facility to outpatient care. The patient may no longer require the 24-hour medical care provided in an inpatient setting, but has continued significant rehabilitation needs. The day rehabilitation program may be an appropriate transition step toward outpatient rehabilitation, in lieu of continued inpatient rehabilitation.